



February 23, 2026

Thomas Keane
Assistant Secretary for Technology Policy
U.S. Department of Health and Human Services
330 C St SW
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Washington, DC 20201

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care

Dear Dr. Keane:

The National Comprehensive Cancer Network® (NCCN®) is pleased to comment on the Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP) and Office of the National Coordinator for Health Information Technology (ONC) Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care as it relates to NCCN's mission of defining and advancing quality, effective, equitable, and accessible cancer care and prevention so all people can live better lives. NCCN will focus our comments on the initiatives we've undertaken thus far to better understand and safely implement artificial intelligence (AI) in cancer care, as well as on the practical uses of AI in oncology. Additionally, NCCN has included responses to specific questions regarding the limitations and risks of AI and recommendations for appropriate oversight and governance of AI.

NCCN Background

As an alliance of 33 leading academic cancer centers in the United States (US) that treat hundreds of thousands of patients with cancer annually, NCCN® is a developer of authoritative information regarding cancer prevention, screening, diagnosis, treatment, survivorship care, palliative care, and supportive care that is widely used by clinical professionals and payers alike. The NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®) are a comprehensive set of guidelines detailing the sequential management decisions and interventions that currently apply to 97 percent of cancers affecting patients in the US.

NCCN imposes strict policies to shield the guidelines development processes from external influences. The "firewall" surrounding the NCCN Guidelines processes includes financial support policies; panel participation and communication policies; guidelines disclosure policies; and policies regarding relationships to NCCN's other business development activities. The guidelines development is supported exclusively by the Member Institutions' dues and does not accept any form of industry or other external financial support for the guidelines development program. The NCCN Guidelines are updated at least annually in an evidence-based process integrated with the expert judgment of multidisciplinary panels of expert physicians from NCCN

NCCN is an alliance of leading cancer centers dedicated to improving and facilitating quality, effective, equitable, and accessible cancer care so all patients can live better lives.

For Clinicians: [NCCN.org](https://www.nccn.org) | For Patients: [NCCN.org/patients](https://www.nccn.org/patients) | Member Institutions: [NCCN.org/cancercenters](https://www.nccn.org/cancercenters)

Member Institutions. The NCCN Guidelines are transparent, continuously updated, available free of charge online for non-commercial use and are available through a multitude of health information technology (HIT) vendors.

NCCN's Approach to AI

NCCN recognizes both the promise and risk posed by the emergence of AI capabilities in healthcare and has actively engaged in work to ensure that AI-driven tools deployed in cancer care does not negatively impact patients. In September 2025, NCCN hosted a policy summit entitled, *The Evolving Artificial Intelligence Landscape in Cancer Care*. The summit provided an opportunity for a varied group of stakeholders including patients, providers, payers, patient advocacy organizations, policy makers, and industry to thoughtfully discuss diverse perspectives related to the integration of AI into cancer care systems and standards. A subsequent policy report is forthcoming and will be publicly available at nccn.org/policy.

Additionally, NCCN is working to identify ways to responsibly optimize and enhance our existing resources for a digital and AI-enabled future. Recognizing the importance of interoperability and standardization for the future, NCCN has been actively working to standardize terminology within our resources and content regarding biomarkers and systemic therapies. The NCCN Guidelines Navigator, based on NCCN's Clinical Practice Guidelines in Oncology in structured data format, serves as a vital tool for healthcare providers, enabling them to quickly find and understand the best available evidence-based treatment options for patients. The platform's intuitive design facilitates easier treatment decision-making by organizing information based on the latest evidence and expert consensus. As the system evolves, the underlying structured data represents a significant opportunity for enhancement to its functionality, including AI-based functionality, all while maintaining the highest standards of data privacy and security. The system also maintains a strong commitment to transparency and accuracy by ensuring that all generated text references the approved NCCN Guidelines, enabling users to review source documents for validation and access additional background information within the NCCN Guidelines.

NCCN is also taking a measured, thoughtful approach to the integration of our content into third-party AI-based tools. NCCN licenses our content for integration into various health information technology-based tools and platforms. To ensure fidelity of the content within these tools, NCCN maintains a rigorous quality assurance process to evaluate the accuracy of how our content is presented. In addition, some of these tools link to our website to ensure easy access to the source information, a critical component of transparent, accurate, responsible AI. NCCN welcomes engagement regarding how best to ensure evidence-based content such as ours can be utilized by AI-based tools in a way that is responsible, secure, transparent, and accurate to the underlying reference sources.

Practical Uses of AI in Oncology

The use of AI in cancer care is growing as new innovations have emerged for use across the care continuum. These innovations support cancer screening and early detection, help predict prognosis, assist providers with healthcare delivery, and serve as a resource for patients to understand their diagnosis.

Diagnosis and Prognosis AI Tools

AI systems are increasingly being trained on large collections of medical images and clinical data to help improve cancer diagnostics and early detection. Because AI can identify subtle changes that may be difficult for the human eye to detect, it has the potential to improve accuracy and support more timely diagnoses.¹ As the software is exposed to more data over time, its performance can continue to improve. Research studies have shown promising results thus far. A study of more than 91,000 mammograms in the United States and United Kingdom found that an AI system reduced false negatives by about 9.4% compared to radiologist interpretation alone.² Another large study of roughly 1.2 million mammograms across Germany and the United States found that radiologists working with AI detected about 2.6% more cancers than radiologists alone.^{3,4} AI is also being explored in other cancer screenings, such as colonoscopy, where studies have found that AI-assisted procedures can increase adenoma detection rates by about 20%.⁵ While AI is not a replacement for providers, these tools show potential in supporting screening programs and improving early detection.

Research has also shown the potential use of AI to go beyond diagnosis and help with predicting prognosis. Researchers at Stanford Medicine developed a multimodal AI model able to incorporate visual and language-based information. After training on 50 million pathology slides, medical images and more than 1 billion pathology-related texts, the model outperformed traditional methods in its ability to predict the prognoses of thousands of people with diverse types of cancer. The algorithm helped identify which people with lung or gastroesophageal cancers are likely to benefit from immunotherapy, and to identify people with melanoma who are most likely to experience a recurrence of their cancer.⁶

There is an active research portfolio looking at ways in which AI can improve screening and early detection. While these examples demonstrate the promise of adding AI to human-based screening tools, further research to optimize the use of AI-inclusive tools and how best to integrate them into the clinical decision-making and workflow are needed to before wholesale integration.

Clinical Trial Matching

AI is also being explored as a tool to improve clinical trial matching in oncology, an area where traditional processes are often time and resource intensive. Identifying eligible patients for trials requires manual review of electronic health records (EHRs), pathology reports, and physician

¹ Ahn, J. S., Shin, S., Yang, S. A., Park, E. K., Kim, K. H., Cho, S. I., Ock, C. Y., & Kim, S. (2023). Artificial Intelligence in Breast Cancer Diagnosis and Personalized Medicine. *Journal of breast cancer*, 26(5), 405–435. <https://doi.org/10.4048/jbc.2023.26.e45>

² McKinney, S.M., Sieniek, M., Godbole, V. et al. International evaluation of an AI system for breast cancer screening. *Nature* 577, 89–94 (2020). <https://doi.org/10.1038/s41586-019-1799-6>

³ Conner, K. (2026, January 29). Using AI to detect breast cancer: What we know. Using AI (Artificial Intelligence) to Detect Breast Cancer. <https://www.breastcancer.org/screening-testing/artificial-intelligence>

⁴ Leibig, C., Brehmer, M., Bunk, S., Byng, D., Pinker, K., & Umutlu, L. (2022). Combining the strengths of Radiologists and AI for Breast Cancer Screening: A retrospective analysis. *The Lancet Digital Health*, 4(7). [https://doi.org/10.1016/s2589-7500\(22\)00070-x](https://doi.org/10.1016/s2589-7500(22)00070-x)

⁵ Makar, J., Abdelmalak, J., Con, D., Hafeez, B., & Garg, M. (2025). Use of artificial intelligence improves colonoscopy performance in Adenoma Detection: A systematic review and meta-analysis. *Gastrointestinal Endoscopy*, 101(1). <https://doi.org/10.1016/j.gie.2024.08.033>

⁶ Xiang, J., Wang, X., Zhang, X., Xi, Y., Eweje, F., Chen, Y., Li, Y., Bergstrom, C., Gopaulchan, M., Kim, T., Yu, K. H., Willens, S., Olguin, F. M., Nirschl, J. J., Neal, J., Diehn, M., Yang, S., & Li, R. (2025). A vision-language foundation model for precision oncology. *Nature*, 638(8051), 769–778. <https://doi.org/10.1038/s41586-024-08378-w>

notes among other important patient data. Cancer clinical trials play a critical role in advancing treatment and improving patient outcomes. NCCN Guidelines emphasize the importance of clinical trials and encourage participation when applicable and available. NCCN has also developed patient resources that provide information about clinical trials and offer guidance on how patients can talk to their providers to determine whether a clinical trial may be a suitable option for them.⁷ Despite the importance of trials for advancing cancer care and most Americans (70%) viewing clinical trial participation favorably, only about 7% of eligible adult cancer patients in the United States ultimately enroll in clinical trials.^{8,9} Emerging AI tools have been developed to help address this gap by rapidly scanning both structured and unstructured data in EHRs to flag patients who may meet complex eligibility criteria and alert care teams in real time. For example, recent 2026 JCO Clinical Cancer Informatics research article described the development and validation of a semi-automated AI-supported clinical trial matching tool that significantly improved recruitment efficiency and scalability, reduced chart review workload roughly ten-fold and shortened screening time by about 41% for patients who continued to manual review.¹⁰ While human oversight remains essential, these tools may help oncology teams connect patients to appropriate trials more quickly and improve trial enrollment. Further evaluation regarding deployment and scaling of these tools into clinical workflows will help harness the potential to improve evaluation and enrollment of patients into cutting-edge trials and advance cancer care.

Clinical Notes

AI-enabled “ambient listening” and digital scribing tools are increasingly being used in oncology to help document appointments and reduce administrative burden on care teams. These systems use speech recognition and natural language processing to listen to conversations, generate draft clinical notes, and populate structured fields in EHRs, allowing providers to focus more on patient interaction rather than typing during or after visits. In cancer care, where visits are often complex, multidisciplinary, and emotionally burdensome, this type of support can be especially valuable. A multiphase study comparing AI ambient documentation with standard EHR documentation in outpatient settings using simulated physician–patient encounters found that AI-assisted documentation significantly improved note quality and reduced consultation time by about 26.3%.^{11,12} The study included clinicians and patient actors across several specialties, including oncology, and demonstrated improved completeness, organization, and clarity of clinical notes when AI support was used.¹¹ Evaluations done by health systems have revealed that AI-generated notes often require editing but can substantially reduce documentation time

⁷ Clinical Trials. National Comprehensive Cancer Network. (n.d.-a). <https://www.nccn.org/patientresources/patient-resources/additional-resources/clinical-trials>

⁸ Kumar, G., Chaudhary, P., Quinn, A., & Su, D. (2022). Barriers for cancer clinical trial enrollment: A qualitative study of the perspectives of healthcare providers. *Contemporary clinical trials communications*, 28, 100939. <https://doi.org/10.1016/j.conctc.2022.100939>

⁹ Unger, J. M., Shulman, L. N., Facktor, M. A., Nelson, H., & Fleury, M. E. (2024). National Estimates of the Participation of Patients With Cancer in Clinical Research Studies Based on Commission on Cancer Accreditation Data. *Journal of clinical oncology : official journal of the American Society of Clinical Oncology*, 42(18), 2139–2148. <https://doi.org/10.1200/JCO.23.01030>

¹⁰ Gong, G., Liu, J., Pandya, S., Taborda, C., Wiesendanger, N., Price, N., Byron, W., Coppi, A., Young, P., Wiess, C., Dunning, H., Barganier, C., Brodeur, R., Fischbach, N., LoRusso, P., Pusztai, L., Kim, S. Y., Rozenblit, M., Cecchini, M., ... Kunz, P. L. (2026). Clinical trial patient matching: A real-time, common data model and artificial intelligence–driven system for semiautomated patient prescreening in cancer clinical trials. *JCO Clinical Cancer Informatics*, (10). <https://doi.org/10.1200/cci-25-00262>

¹¹ Afzal, M., French, K. E., Bilbrey, L. E., & Faruki, A. A. (2025). Artificial Intelligence in the clinic: Creating harmony or just adding noise? *American Society of Clinical Oncology Educational Book*, 45(3). <https://doi.org/10.1200/edbk-25-481490>

¹² Balloch, J., Sridharan, S., Oldham, G., Wray, J., Gough, P., Robinson, R., Sebire, N. J., Khalil, S., Asgari, E., Tan, C., Taylor, A., & Pimenta, D. (2024). Use of an ambient artificial intelligence tool to improve quality of clinical documentation. *Future Healthcare Journal*, 11(3), 100157. <https://doi.org/10.1016/j.fhj.2024.100157>

and cognitive load, helping address burnout among providers.¹³ As these tools are rapidly being integrated into clinical workflows, optimizing them to capture clinically-relevant information reflective of the clinical encounter will be important for long-term utility.

Responses to Specific Questions

In addition to providing an overview of NCCN's approach to AI and some of the practical applications of AI in oncology, NCCN has included responses to specific questions outlined in the RFI that were not otherwise addressed above

What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Despite the practical use of AI-driven tools in cancer care, these tools can also pose risks to patients if they are developed and used without appropriate guardrails and oversight. These concerns leave many health systems and providers apprehensive about the adoption of AI in clinical care. Specific concerns are detailed below.

Data Protection

AI tools rely heavily on large volumes of sensitive health data, making privacy and data protection a major concern. While HIPPA establishes safeguards for patient information, uncertainty remains around how data is collected, stored and shared, especially when the data may be accessed by third-party vendors. Major privacy concerns include unauthorized access to patient data through breaches or cyberattacks, as well as the potential misuse of data without adequate transparency or oversight.^{14,15} As AI tools often rely on cloud-based infrastructure and data sharing across institutions, concerns about security vulnerabilities and lack of oversight have heightened.

Transparency and Trust

Transparency is also a major concern in the use of AI systems in healthcare. Health systems and providers may lack clear documentation describing how an AI tool was developed or tested because of limited use of model cards and performance reports that would allow users to understand the limitations and risks. Without standardized documentation, it is harder to evaluate safety and effectiveness.¹⁶

Heightening this challenge, many AI systems function as “black box” models, meaning they produce outputs without clearly explaining how or why a decision or recommendation is

¹³ Olson, K. D., Meeker, D., Troup, M., Barker, T. D., Nguyen, V. H., Manders, J. B., Stults, C. D., Jones, V. G., Shah, S. D., Shah, T., & Schwamm, L. H. (2025). Use of ambient AI scribes to reduce administrative burden and professional burnout. *JAMA Network Open*, 8(10). <https://doi.org/10.1001/jamanetworkopen.2025.34976>

¹⁴ Price, W. N., & Cohen, I. G. (2019). Privacy in the age of Medical Big Data. *Nature Medicine*, 25(1), 37–43. <https://doi.org/10.1038/s41591-018-0272-7>

¹⁵ Di Palma, G., Scendoni, R., Ferorelli, D., De Benedictis, A., Tambone, V., & De Micco, F. (2025). AI-Induced Cybersecurity Risks in Healthcare: A Narrative Review of Blockchain-Based Solutions Within a Clinical Risk Management Framework. *Risk management and healthcare policy*, 18, 3479–3497. <https://doi.org/10.2147/RMHP.S544523>

¹⁶ Blueprint for Trustworthy AI Implementation Guidance. Coalition for Health AI. (2023, April 4).

https://assets.ctfassets.net/7s4afyr9pmov/4AXIWGllcrjWDaW2ueTaRS/t98e5cb2528187635895cce6ba5ec309/Blueprint_for_Trustworthy_AI.pdf

generated.¹⁷ This lack of explainability can hinder a provider's ability to assess the reliability of AI-generated responses and integrate them responsibly into decision making. Additional concerns about trust are exacerbated by the risks of AI "hallucinations". AI hallucinations occur when AI-driven tools perceive patterns or objects that are nonexistent or undetectable to human observers, creating outputs that are nonsensical or altogether inaccurate.¹⁸ In healthcare, such errors are particularly concerning, as they may introduce misinformation into workflows and potentially contribute to inappropriate decision making that may negatively impact patients.

The lack of transparency with AI-driven tools can undermine trust among patients and providers, complicate clinical decision making, and limit meaningful informed consent if patients and providers cannot understand the basis of AI-generated recommendations. A 2024 Wolters Kluwer Health survey of 100 U.S. physicians revealed that 91% of respondents indicated they need to know that generative AI materials were created by doctors and medical experts before using them in clinical decisions.¹⁹ Similarly, 89% reported that they would be more likely to use generative AI in clinical decision making if a vendor was transparent about where information came from, who created it, and how it was sourced. The lack of transparency makes accountability difficult. Without oversight and standards for explainability and transparency, the use of AI in cancer care may challenge existing privacy protection frameworks, patient autonomy, and ethical use of data.

Prior Authorization

Health plans' use of AI decision making tools in prior authorization processes raises significant concerns for patients and providers. While insurers often describe prior authorization as a mechanism to control costs by preventing unnecessary, high-cost, or inappropriate care, the process can be burdensome for providers and create delays or barriers to timely, comprehensive care.²⁰ The integration of AI into these systems risks further exacerbates these concerns especially when AI is used to flag or deny requests without sufficient clinical context, transparency, or meaningful human review. For patients, this can translate into delayed treatment, additional appeals, or confusion about how coverage decisions are made. For cancer patients specifically, research demonstrates that even short delays in cancer treatment are associated with increased mortality risk. For example, a one-month delay in cancer surgery can result in a 6 percent to 8 percent increase in the risk of death.²¹ In breast cancer specifically, an eight-week delay in breast cancer surgery increased the risk of death by 17 percent, while a 12-week delay increased the risk by 26 percent.²¹ These concerns are also reflected in 2025 physician survey data from the American Medical Association, in which 61% of physicians reported being

¹⁷ Xu, H., & Shuttleworth, K. M. (2024). Medical Artificial Intelligence and the Black Box Problem: A view based on the ethical principle of "Do no harm." *Intelligent Medicine*, 4(1), 52–57. <https://doi.org/10.1016/j.imed.2023.08.001>

¹⁸ IBM. (2025, November 17). What are ai hallucinations?. IBM. <https://www.ibm.com/think/topics/ai-hallucinations>

¹⁹ Generative AI in Healthcare: Growing Physician Enthusiasm. Wolters Kluwer. (2024, April). <https://assets.contenthub.wolterskluwer.com/api/public/content/2231209-gen-ai-executive-brief-pdf?v=09b080b4>

²⁰ Pickern, J. S. (2025, April 2). Prior authorizations and the adverse impact on continuity of care. *AJMC*. <https://www.ajmc.com/view/prior-authorizations-and-the-adverse-impact-on-continuity-of-care>

²¹ Hanna, T. P., King, W. D., Thibodeau, S., Jalink, M., Paulin, G. A., Harvey-Jones, E., O'Sullivan, D. E., Booth, C. M., Sullivan, R., & Aggarwal, A. (2020). Mortality due to cancer treatment delay: Systematic review and meta-analysis. *BMJ*, m4087. <https://doi.org/10.1136/bmj.m4087>

concerned that health plans' use of AI is increasing prior authorization denials, exacerbating avoidable patient harms, and escalating unnecessary waste now and into the future.²²

Bias in AI Algorithms

Research has shown that AI systems can perpetuate bias at various stages of the development process, including data collection, model training, and real-world implementation.²³ Because AI models are trained on existing datasets, they may not adequately represent the full population for whom they are intended. An algorithm built on limited data can perpetuate false assumptions and biases in the medical field, impacting patient care and worsening healthcare disparities.²⁴ For example, AI prediction models used for melanoma diagnosis exhibit worse performance for images of lesions in darker skin tones because the majority of training datasets for these models consist of images from fair-skinned populations.²⁵ This is extremely worrisome because at time of diagnosis, darker-skinned patients already present with later stages of the disease and have lower survival rates than fair-skinned patients.²⁵ Additional sources of bias in AI software can stem from difference in experience and expertise among the individuals involved in developing these systems, as well as environmental bias, empathy bias, and evidence bias, all of which may influence how these tools are designed, trained, and implemented in practice.²⁶

Although awareness of bias in AI systems is well documented, it is not being addressed consistently across healthcare. A national analysis conducted by the University of Minnesota School of Public Health examined how hospitals use and evaluate these tools. While 61% of hospitals reported they evaluated their predictive models for accuracy, only 44% conducted similar evaluations for bias.²⁷ These findings underscore the need for stronger, more consistent oversight to ensure that AI used in cancer care and healthcare more broadly is safe, equitable, and effective for all patients.

Training and Workforce Preparedness

As AI tools are increasingly integrated into healthcare systems, it is essential that the healthcare workforce receives appropriate training to ensure these tools are used safely and effectively. Providers must be trained to not only operate AI tools but must also understand how to interpret AI-generated outputs and identify their limitations. Without comprehensive and ongoing training there are risks of misuse, overreliance, or inappropriate application of AI tools in patient care.

Despite these risks, workforce training has not kept pace with the rapid implementation of AI across healthcare settings. A systematic literature review examining barriers to successful implementation of AI-driven technologies in healthcare identified 92 relevant studies, more than one-third (33) of the which cited lack of training as a concern or challenge to adoption among

²² AMA State Advocacy Impact Report. American Medical Association. (2026, January 8). <https://www.ama-assn.org/health-care-advocacy/state-advocacy/ama-state-advocacy-impact-report>

²³ Cross, J. L., Choma, M. A., & Onofrey, J. A. (2024). Bias in medical AI: Implications for clinical decision-making. *PLOS digital health*, 3(11), e0000651. <https://doi.org/10.1371/journal.pdig.0000651>

²⁴ Hurd, T. C., Cobb-Payton, F., & Hood, D. B. (2024). Targeting machine learning and artificial intelligence algorithms in health care to reduce bias and improve population health. *The Milbank Quarterly*, 102(3), 577–604. <https://doi.org/10.1111/1468-0009.12712>

²⁵ Adamson AS, Smith A. Machine Learning and Health Care Disparities in Dermatology. *JAMA Dermatol*. 2018;154(11):1247–1248. doi:10.1001/jamadermatol.2018.2348

²⁶ Centers for Disease Control and Prevention. (2024b, August 22). Health equity and ethical considerations in using Artificial Intelligence in public health and medicine. Centers for Disease Control and Prevention. https://www.cdc.gov/pcd/issues/2024/24_0245.htm

²⁷ Nong, P., Adler-Milstein, J., Apathy, N. C., Holmgren, A. J., & Everson, J. (2025). Current use and evaluation of artificial intelligence and predictive models in US hospitals. *Health Affairs*, 44(1), 90–98. <https://doi.org/10.1377/hlthaff.2024.00842>

healthcare providers.²⁸ Similarly, a study of 501 U.S. clinicians found that only 42% of them reported a sense of preparedness for AI implementation and 50% acknowledged the potential need for retraining to effectively utilize generative AI.²⁹ As we identify ways to accelerate innovation, it is critically important to ensure users of these systems are adequately prepared and competent to prevent unintended harm to patients. These findings underscore a gap between the technological advancements of AI and workforce preparedness. As we seek ways to accelerate innovation and expand the use of AI in oncology and other areas of care, equal attention must be given to ensuring providers are adequately trained and supported.

How can HHS best support private sector activities (e.g., accreditation, certification, industry driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Engaging with and utilizing resources from evidence-based organizations like NCCN should be a priority for HHS to advance innovative and effective AI use in clinical care. NCCN Guidelines[®] and Library of Compendia products help ensure access to appropriate care, clinical decision-making, and assessment of quality improvement initiatives. The NCCN Drugs & Biologics Compendium (NCCN Compendium[®]) has been recognized by CMS and clinical professionals in the commercial payer setting since 2008 as an evidence-based reference for establishment of coverage policy and coverage decisions regarding off-label use of anticancer and cancer-related medications. NCCN was recognized by CMS in 2016 and renewed in 2021 as a qualified Provider Led Entity (PLE) for the Medicare Appropriate Use Criteria (AUC) Program for the development of AUC and the establishment of policy and decision-making for diagnostic imaging in patients with cancer.

These recognitions underscore the reliability and rigor of NCCN Guidelines and resources as authoritative tools for cancer care and prevention. As private sector developers increasingly build AI-driven tools for clinical care, it is imperative that the data and content used to train and inform these systems are grounded in high-quality evidence-based resources. Engaging with trusted, evidence-based organizations like NCCN can help ensure that AI clinical resources deliver accurate, trustworthy, reliable information and improve patient outcomes.

What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have.

Patients have begun turning to AI tools to better understand medical information, prepare for appointments, and generate questions to discuss with their providers. Conversational AI systems can explain diagnoses, treatments, and test results in plain language and can engage in extended dialogue, giving patients more time to process complex information and identify concerns they want to raise with providers. Surveys suggest this use is growing. A Kaiser Family Foundation

²⁸ Abdelwanis, M., Simsekler, M. C. E., Gabor, A. F., Sleptchenko, A., & Omar, M. (2026). Artificial intelligence adoption challenges from healthcare providers' perspectives: A comprehensive review and future directions. *Safety Science*, 193, 107028. <https://doi.org/10.1016/j.ssci.2025.107028>

²⁹ ClinicAI Companions Report: Clinician's perspective on Generative AI and the future of healthcare. QuestionPro. (2024). <https://www.questionpro.com/ebook/clinicai-companions-report/>

survey reported that about one in six adults (17%) uses AI chatbots at least monthly for health advice or information, and an April 2025 Annenberg Public Policy Center health survey of more than 1,600 U.S. adults found that nearly two-thirds (63%) viewed AI-generated health information as somewhat or very reliable.^{30,31} While these technologies are not a substitute for professional medical advice and can sometimes produce incomplete or inaccurate information, they may help patients feel more informed and prepared more effectively for clinical encounters.

At the same time, increased patient use does not always necessarily translate into broad confidence in AI's role within healthcare delivery. Similar to providers, many patients remain cautious about the use of AI in healthcare delivery. A study published in *JAMA Network* found that 65.8% of surveyed US adults reported low trust in their health care system to use AI responsibly and 57.7% expressed low trust that their health care system would make sure an AI tool would not harm them.³² Similarly, a national survey of a total of 926 respondents across the US examining the public perception of the use of AI in diagnosis and treatment found respondents were very concerned or somewhat concerned about AI's unintended consequences, including misdiagnosis (91.5%), privacy breaches (70.8%), less time with clinicians (69.6%), and higher health care costs (68.4%)³³ These concerns continue to highlight the need for appropriate oversight, transparency, and guardrails that consider patients to ensure safe integration of AI in healthcare.

Recommendations

NCCN urges ASTP/ONC to consider mechanisms for appropriate regulation of AI within healthcare, which poses unique opportunities and risks for patient care, patient safety, and patient outcomes. NCCN notes that the National Academy of Medicine developed an AI Code of Conduct to provide a unifying framework to guide responsible, equitable, and human-centered AI. The guiding principles are as follows:³⁴

1. **Engaged-** Center AI development on people's needs, preferences, and goals throughout its lifecycle.
2. **Safe-** Maintain strict oversight to prevent and address any potential harm from AI in health and medicine.
3. **Effective-** Ensure AI consistently improves health and well-being while upholding ethical standards.
4. **Equitable-** Demonstrate fairness in AI design, access, and outcomes, minimizing bias and risk.

³⁰ Presiado, M., Hamel, L., Lopes, L., & Montero, A. (2025, August 13). KFF Health Misinformation Tracking Poll: Artificial Intelligence and Health Information. KFF. <https://www.kff.org/public-opinion/kff-health-misinformation-tracking-poll-artificial-intelligence-and-health-information/#cdc7ee85-54db-4b29-aa9b-625afeb4c03c>

³¹ Many in U.S. consider AI-generated health information useful and reliable. The Annenberg Public Policy Center of the University of Pennsylvania. (2025, October 21). <https://www.annenbergpublicpolicycenter.org/many-in-u-s-consider-ai-generated-health-information-useful-and-reliable/>

³² Nong, P., & Platt, J. (2025). Patients' Trust in Health Systems to Use Artificial Intelligence. *JAMA network open*, 8(2), e2460628. <https://doi.org/10.1001/jamanetworkopen.2024.60628>

³³ Khullar, D., Casalino, L. P., Qian, Y., Lu, Y., Krumholz, H. M., & Aneja, S. (2022). Perspectives of Patients About Artificial Intelligence in Health Care. *JAMA network open*, 5(5), e2210309. <https://doi.org/10.1001/jamanetworkopen.2022.10309>

³⁴ Health Care Artificial Intelligence Code of conduct. NAM. (2025, December 1). <https://nam.edu/our-work/programs/leadership-consortium/health-care-artificial-intelligence-code-of-conduct/>

5. **Efficient-** Use AI to improve health outcomes while reducing resource use and environmental impact.
6. **Accessible-** Make every stage of AI development and governance open and inclusive for all stakeholders.
7. **Transparent-** Share clear, understandable information about how AI works, performs, and impact outcomes.
8. **Secure-** Protect health data through validated privacy and security measures that enable safe, continuous learning.
9. **Adaptive-** Continuously monitor and refine AI systems to drive ongoing learning and improvement in health and science.

NCCN agrees with and supports these principles and believes they offer important guidance for ASTP/ONC as it considers regulations related to healthcare-related applications of AI. Without comprehensive federal legislation or regulation, responsibility for oversight is left to states, local governments, and individual health systems. This fragmented approach can be costly and resource-intensive, and has resulted in uneven AI implementation across the country, with lower access rates in areas that already exhibit high healthcare access needs and where existing, proven AI tools could be implemented.³⁵ As noted in the Executive Order titled Ensuring A National Policy Framework For Artificial Intelligence, a national standard and framework is needed. As this framework is developed, NCCN strongly encourages ASTP/ONC to establish clear standards that support innovation while also prioritizing safety, transparency, and accountability, so that AI is developed responsibly without causing harm to patients. Further, engagement with healthcare stakeholder organizations across the spectrum will be critical to inform effective policies that serve the best interests of patients.

NCCN appreciates the opportunity to comment on ASTP/ONC Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care. NCCN is happy to serve as a resource and looks forward to working together to advance access to equitable, high-quality cancer care.

Sincerely,



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³⁵ Hwang, YM., Ng, M.Y., Pillai, M. et al. The landscape of AI implementation in US hospitals. Nat. Health 1, 99–112 (2026).
<https://doi.org/10.1038/s44360-025-00016-7>